

Guideline

Treatment of the acute management of status epilepticus in children (1 month - 16 years of age)

1 Scope

For use within children's services and the paediatric emergency department.

2 Purpose

To provide guidance on the management of status epilepticus in children.

3 Introduction

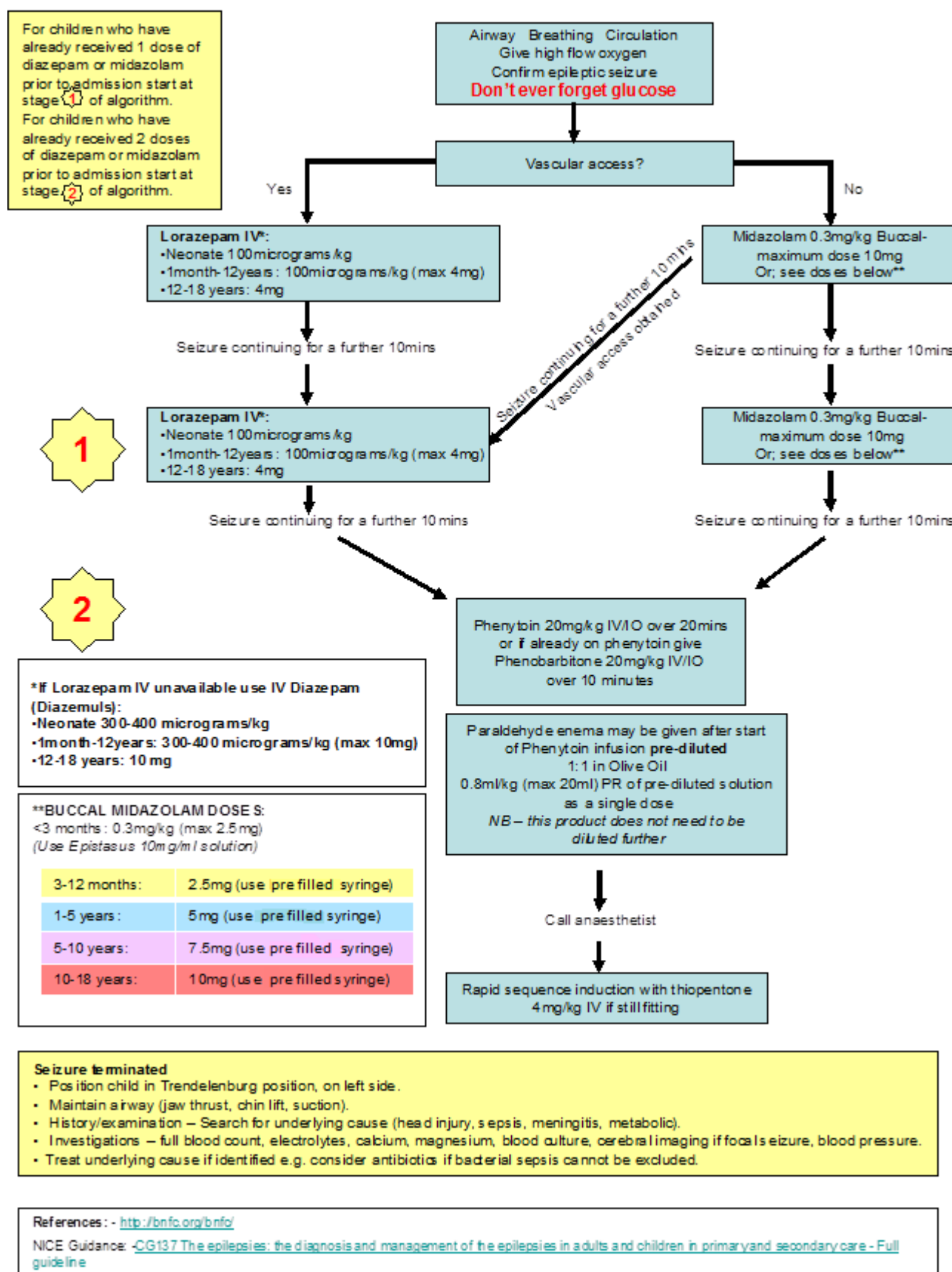
This guideline has been developed:

- to incorporate new research regarding improved management of children with buccal midazolam rather than rectal diazepam
- to provide guidance for the management of status epilepticus in line of latest NICE recommendations.

4 Treatment guidelines for the acute management of status epilepticus in children (1 month-16 years)

See overleaf.

Treatment guidelines for the acute management of Status Epilepticus in Children (1 month - 16 years of age)



5 Confirmation of epileptic seizures

A guide to **expertise** in this area is outside the scope of this document.

However, it should be noted that treatment overleaf has significant morbidity. Not all shaking movements are epileptic. When both sides of the body are convulsing due to an epileptic seizure, consciousness will be lost. Where it is maintained, particularly in children over the age of eight years, it is essential to consider whether this is a psychologically dependent paroxysmal episode, **prior** to treatment being administered. Convulsive status epilepticus is always associated with lethargy and disorientation for several hours afterwards. Where the child/ young person normalizes within five minutes, the episode is not likely to have been convulsive status epilepticus.

6 Hypoxia

It is common for children to become blue, ie hypoxic, during a seizure. Start high flow oxygen on children in convulsive status epilepticus in the hospital, unless previously directed otherwise. Nevertheless, the child may remain blue ie hypoxic. Although concerning, the best solution to the hypoxia is cessation of the seizure and appropriate airway management at that point. However, if the child develops bradycardia and reliable oxygen saturations continue to decline beneath 80%, then urgent anaesthetic review should be initiated.

7 Monitoring compliance with and the effectiveness of this document

Children's services will audit the standards in this document periodically. The audit standards used will be:

- Initial management of status epilepticus on admission in line with guidelines
- Doses and timings of medication given as per guidelines

All audits will be registered with the Trust clinical governance department. All results will be reported to clinical governance.

8 References

BNF for Children 2016-2017

<https://www.medicinescomplete.com/mc/bnfc/current/>

NICE Guidance: CG137 The epilepsies: the diagnosis and management of the epilepsies in adults and children in primary and secondary care - Full guideline

<https://www.nice.org.uk/guidance/CG137>

Equality and diversity statement

This document complies with the Cambridge University Hospitals NHS Foundation Trust service equality and diversity statement.

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Document management

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File name:	Status epilepticus in children Version5 January 2017.doc		
Supersedes:	Version 4, April 2014		
Version number:	5	Review date:	January 2020
Local reference:		Document ID:	1172